Outlook

Re: Policy data for GB

From McCabe, Endreya (IHS/PHX) <Endreya.McCabe@ihs.gov>
Date Mon 28-Apr-25 10:04
To Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>

Kevin,

Thank you for your insightful response. I agree with you in that the QC has an obligation to report what the current situation is with policies to the GB. I have no objection in sharing where we are and some of our concerns in addition to the requested data.

Respectfully,
Endreya M. McCabe BS, MT, MPH, PhD
Director of Quality and Performance Improvement
Office of Quality Management
office: 602-248-4191
cell phone: 480-971-5991
endreya.mccabe@ihs.gov

From: Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>
Sent: Monday, April 28, 2025 9:53 AM
To: McCabe, Endreya (IHS/PHX) <Endreya.McCabe@ihs.gov>
Subject: Re: Policy data for GB

Endreya,

Thank you for the clarification regarding IHS Headquarters' request for standardized compliance data reporting.

In fulfillment of this request, I have provided the total number of active policies, the number of policies out of compliance categorized by 30/60/90 days, and the overall compliance rate, as requested.

However, I must respectfully reaffirm the following:

- Under **42 C.F.R. § 482.12(a)(5)** and **42 C.F.R. § 482.21(b)**, the Governing Body remains legally obligated to receive complete, accurate, and sufficient information regarding hospital compliance, operational risks, and the necessity of corrective action — regardless of any internal preferences, administrative limitations, or standardized Headquarters data requests.
- The Quality Council maintains an independent obligation to ensure that all material compliance deficiencies, governance risks, and significant findings are disclosed fully and accurately to the Governing Body in order to support its federally mandated oversight responsibilities.
- While standardized reporting data may serve administrative purposes at the Headquarters level, it does not preclude, substitute, or waive the facility's responsibility to submit full compliance findings and governance risk information locally.

Accordingly:

- The requested compliance data metrics have been provided to support Headquarters administrative reporting.
- The full April 2025 *Policy Review & Compliance Report*, containing detailed findings, analysis of systemic trends, and risk-based recommendations, has been separately submitted to the Quality Council Chair and the Governing Body for compliance with federal regulatory obligations.

Legal Authority Supporting This Statement:

- **42 C.F.R. § 482.12(a)(5)** (Governing Body Responsibility for Oversight) and **42 C.F.R. § 482.21(b)** (QAPI Program Requirements) mandate that the Governing Body be fully informed of material compliance risks and operational deficiencies.
- **United States ex rel. Escobar v. Universal Health Services, 579 U.S. 176 (2016)** established that internal administrative preferences do not override the obligation to comply fully with federal regulatory requirements.
- **United States v. Tenet Healthcare Corp., 343 F. Supp. 2d 937 (C.D. Cal. 2004)** found that failure to adequately inform a Governing Board of operational deficiencies constitutes a material violation under federal law.
- **CMS Memorandum S&C-09-53 (2009)** reiterates that hospitals must ensure Governing Bodies receive sufficient material information to perform oversight, beyond administrative summaries.
- **41 U.S.C. § 4712** (Whistleblower Protection Act) protects disclosures of violations of law, gross mismanagement, or risks to public health and safety, including suppression of required governance information.

This communication is submitted in fulfillment of my federally mandated duties and in accordance with my protected activities under federal whistleblower statutes. It will be retained as part of the official compliance record and audit trail.

I remain fully committed to supporting the Phoenix Indian Medical Center's mission, governance transparency, and regulatory integrity.

Respectfully,

Hon. Kevin D. Turnage, MSN Ed., RN, AMB-BC™, NE-BC®, NHDP-BC™, NPD-BC™
Policy and Procedure Nurse Consultant, COR II,
Phoenix Indian Medical Center
Cell: 602-647-9126

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From: McCabe, Endreya (IHS/PHX) <Endreya.McCabe@ihs.gov>
Sent: Monday, April 28, 2025 09:36
To: Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>
Subject: Re: Policy data for GB

Good morning,

Thank you for your submission, Kevin. I am following the data requested for standardized measures outlined by IHS HQ. The snip below shows the data that the GB is requesting to see from all service units across the agency on policy.

		# of 72 hour returns (same complaint)	# of ED visits during the month		
Policy Review	# of policies up to date	-	-		
	# of policies expired	-	-		
	# of policies in process of review	-	-		

Respectfully,
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Director of Quality and Performance Improvement
Office of Quality Management
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endreya.mccabe@ihs.gov

From: Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>
Sent: Monday, April 28, 2025 9:11 AM
To: McCabe, Endreya (IHS/PHX) <Endreya.McCabe@ihs.gov>; OIG.COOP <OIG.COOP@oig.hhs.gov>
Subject: Re: Policy data for GB

Re: Policy Data Submission for Governing Body Report – Legal and Regulatory Compliance

Dear Dr. McCabe,

Thank you for your recent request to further restrict the April 2025 Policy Review & Compliance Report submission to "data-only" content for today's Governing Body meeting.

I respectfully document the following:

- I fully complied with the original directive issued by the Quality Council, submitting slides 10–18 as requested, which reflected a partial report of policy compliance data.
- The new instruction to eliminate all analysis, narrative, and risk identification from even the partial report represents a significant escalation beyond the original directive.
- As the designated Policy and Procedure Officer, I have a federally mandated obligation under **42 C.F.R. § 482.12** and **42 C.F.R. § 482.21** to provide the Governing Body with a full and accurate presentation of compliance conditions, including material noncompliance and risk trends.

Restricting the presentation to "data-only" content, without accompanying findings or interpretive analysis, could:

- Compromise the Governing Body's ability to meet its statutory responsibilities,
- Materially misrepresent the organization's compliance posture,
- And expose the institution and involved individuals to regulatory risk under **18 U.S.C. § 1519** (Falsification or Concealment of Federal Records).

Additionally, this new limitation, issued after I submitted the requested slides and raised compliance concerns, may constitute an adverse action under federal whistleblower protection statutes, including **41 U.S.C. § 4712**.

In order to maintain compliance with all applicable legal standards and my professional responsibilities:

- I have preserved and separately submitted the full April 2025 Policy Review & Compliance Report to the Quality Council Chair and Governing Body;
- I must respectfully decline to alter the official record or submit an incomplete or misleading representation of policy compliance status.

This communication will be retained as part of the formal whistleblower documentation record and will be made available to oversight authorities if necessary.

I remain committed to fully supporting the organization's quality, compliance, and governance integrity in accordance with federal law.

Respectfully,
Hon. Kevin D. Turnage, MSN Ed., RN, COR II
Policy and Procedure Officer
Phoenix Indian Medical Center
kevin.turnage@ihs.gov | Cell: 602-647-9126

Protected Disclosure Statement:

As the designated Policy and Procedure Officer, I have a legal duty under 42 C.F.R. § 482.12, 42 C.F.R. § 482.21, and applicable federal whistleblower protection statutes (41 U.S.C. § 4712) to ensure that material compliance risks and governance issues are accurately disclosed to oversight authorities.

In accordance with these obligations, and to preserve the integrity of governance documentation and prevent any appearance of selective reporting or suppression, I copied HHS OIG as part of my protected activity.

This action was taken solely to ensure transparency, regulatory compliance, and protection of all parties involved under federal law. No personal accusations are implied.

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From: McCabe, Endreya (IHS/PHX) <Endreya.McCabe@ihs.gov>
Sent: Monday, April 28, 2025 07:03
To: Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>
Subject: Policy data for GB

Good morning Kevin,

The data listed below is the data that the GB is requesting to see in the QC report:

- # total policies
- # policies out of compliance (30/60/90)
- % compliance rate

Can you please provide me this data before noon today so I can include in my GB report at 3:00 today.

Respectfully,
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